

1. Administrative & Study Identification

Full Study / Official Title: A Phase 3, Randomized, Double-blind Trial of Nivolumab in Combination with Intravesical BCG versus Standard of Care BCG Alone in Participants with High-risk Non-muscle Invasive Bladder Cancer That Is Persistent or Recurrent After Treatment with BCG **Brief Title:** A Study Comparing the Efficacy and Safety of Nivolumab in Combination With Bacillus Calmette-Guerin (BCG) Versus BCG Alone in Participants With High-Risk Non-Muscle Invasive Bladder Cancer (HR NMIBC) **Short Title/ Acronym:** CheckMate 7G8 **Protocol Number:** CA209-7G8 **NCT Number:** NCT04149574 **Sponsor/Company Name:** Bristol Myers Squibb **Investigational Product:** Nivolumab (Drug Name: nivolumab; Trade Names: Opdivo, Opdualag; ATC Code: L01FF01) and Bacillus Calmette-Guerin (BCG) **Disease Areas/Conditions:** Urinary Bladder Neoplasms (Preferred UMLS Name: Non-Muscle Invasive Bladder Cancer) **Phase of Development:** PHASE3 **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective & Brief Summary **Brief Summary:** A study comparing nivolumab and bacterial drugs given to help the body's immune system in the bladder versus bacterial drugs alone in high risk bladder cancer participants. **Primary Objective:** To evaluate if nivolumab in combination with intravesical BCG delays disease worsening (progression) or recurrence compared to standard of care BCG alone in participants with high-risk Non-Muscle Invasive Bladder Cancer (NMIBC). **Secondary Objectives:** To assess overall survival (OS), worsening-free survival, complete response rate (CRR), safety, and tolerability of the combination therapy.

2.2 Background & Rationale To address the unmet medical need in patients with high-risk NMIBC whose disease persists or recurs after standard Bacillus Calmette-Guérin (BCG) treatment. Radical cystectomy is the standard option upon BCG failure but is associated with high morbidity and impaired quality of life. Combining BCG with nivolumab (an immune checkpoint inhibitor) is hypothesized to synergistically stimulate the body's immune system to attack cancer cells, providing an effective bladder-sparing strategy and delaying disease progression.

3. Study Design & Methodology

3.1 Design Framework Study Type: INTERVENTIONAL Control Type: Placebo-controlled (Active Comparator - standard of care BCG + matching placebo) **Blinding:** Double-blind **Randomization:** Randomized (Parallel Assignment) **Trial Status:** TERMINATED (Study was terminated early)

3.2 Planned Enrollment & Duration Target N\$: 13 (Note: Trial was terminated early) **Number of Sites:** Multicenter/Global **Estimated Study Start:** 15-Jan-2020 **Estimated Primary Completion:** 31-Oct-2023 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Persistent or recurrent disease $\leq$ 24 months of last BCG dose, but not classified as BCG unresponsive. 2. Histologically confirmed persistent or recurrent high-risk non-muscle-invasive urothelial carcinoma (UC). 3. Treated with at least 1 adequate course of induction BCG therapy (at least 5 out of 6 doses). 4. Eastern Cooperative Oncology Group (ECOG) performance status (PS) 0-2. 5. Other protocol-defined inclusion/exclusion criteria apply.

4.2 Exclusion Criteria 1. Previous or concurrent muscle invasive, locally advanced, or disseminated/metastatic UC. 2. UC in the upper genitourinary tract (kidneys, renal collecting systems, ureters) within 24 months of enrollment. 3. UC and/or CIS in the prostatic urethra within 12 months of enrollment. 4. Other protocol-defined inclusion/exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Nivolumab (or matching placebo) administered every 4 weeks. Intravesical BCG administered weekly for 6 weeks at the beginning (induction), then weekly for 3 weeks at months 3, 6, 12, 18, 24, 30, and 36 (maintenance). **Route of Administration:** Intravenous (IV) for Nivolumab/Placebo; Intravesical for BCG. **Duration of Treatment:** Up to approximately 36 months of exposure.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for symptoms. **Prohibited:** Concurrent systemic immunosuppressive therapies, or other investigational anti-cancer agents.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent,

Histological Confirmation, Baseline Imaging | | **Baseline** | Day 0
| Randomization, First IV/Intravesical Doses | | **Interim** | Every
4 weeks | IV Nivolumab/Placebo administration, Safety Labs,
Efficacy Assessment (Cystoscopy periodically) | | **End of Study** |
Month 36 | Final assessments, Safety Follow-up, Survival Follow-
up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint * Part 2: Event Free Survival: The time between the date of randomization and the date of first documented event or death due to any cause, whichever occurs first. Events include recurrence of disease (TaHG, T1, or CIS) and progression of disease. * **Measurement Method:** Clinical assessment, cystoscopy, and central histology/biopsy review. * *(Note: Data Not Collected (DNC) for some variables following the termination status of the study.)*

7.2 Secondary & Exploratory Endpoints * Part 2: Worsening-Free Survival * Part 2: Overall Survival: Time from randomization to death due to any cause. * **Part 2: Complete Response Rate at 13 Weeks** * Time to progression to muscle-invasive disease (i.e., T2), cystectomy, systemic chemotherapy, or radiotherapy.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Defined as any new untoward medical occurrence or worsening of a preexisting medical condition. Grading based on NCI CTCAE version 5.0 criteria. **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline for severe/life-threatening events. **Data Monitoring Committee (DMC):** Independent safety monitoring implemented by the sponsor.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes (EFS). Safety Set for all participants receiving at least one dose of the study drug. **Sample Size Justification:** Initially structured for a larger cohort, but the targeted enrollment recorded was N=13. Study was terminated prematurely. **Handling of Missing Data:** Participants who did not experience an event (recurrence or progression) or death were censored at the date of their last evaluable disease assessment or date last known alive.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring of electronic Case Report Forms (eCRFs). **Audit Trail:** Robust electronic data capture with complete audit trails for all data entry, modifications, and central pathology reviews.

11. Study Identifiers & Governance

Primary ID: CA209-7G8 **Posting ID:** 422035838703589492 **Registry Number:** NCT04149574 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** CheckMate 7G8 **Official Regulatory Title:** A Phase 3, Randomized, Double-blind Trial of Nivolumab in Combination with Intravesical BCG versus Standard of Care BCG Alone in Participants with High-risk Non-muscle Invasive Bladder Cancer That Is Persistent or Recurrent After Treatment with BCG

12. Clinical Framework (NMIBC Specific)

Primary Condition: High-Risk Non-Muscle Invasive Bladder Cancer (HR NMIBC) / Urinary Bladder Neoplasms **Diagnosis Threshold:** Histologically confirmed persistent or recurrent disease after ≥ 1 adequate course of BCG **Medical Methodology:** Systemic Immune Checkpoint Inhibition (PD-1 blockade) combined with Local Intravesical Immunotherapy **Optimization Target:** Delayed disease progression, recurrence-free survival, and avoidance of radical cystectomy

13. Study Procedures & Methodology

Management Pathway: BCG induction and maintenance combined with IV immune checkpoint inhibitor **Education Protocol (HCP):** Training on protocol-specific administration schedules, double-blinding procedures, and immune-mediated adverse event management. **Education Protocol (Patient):** Informed consent processes regarding experimental immunotherapy, IV infusions, and intravesical instillation requirements. **Quality Audit Mechanism:** Centralized independent review of cystoscopic findings and pathology to confirm response or progression.

14. Observation & Follow-Up Schedule

Total Duration: Up to 36 months of active treatment + long-term survival follow-up **Onsite Visit Cadence:** Every 4 weeks for IV therapy; weekly during active BCG induction/maintenance blocks **Remote Monitoring Frequency:** Continuous safety and survival

monitoring as defined by the protocol **Checklist Review Interval:** Scheduled periodic cystoscopic evaluations (e.g., every 3 months in the first year)

15. Sign-off & Approval

Role	Name	Signature	Date	---	---	---	---
Principal Investigator	[Insert Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Insert Name]	_____	[DD-MMM-YYYY]				
Lead Statistician	[Insert Name]	_____	[DD-MMM-YYYY]				